

Care at the end of life

The terminally ill patient, palliative care and hospice, euthanasia and physician-assisted suicide

Practical Class 5 · Social Medicine and Medical Ethics I · 2026/2027

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Five questions for today's class

1. The terminally ill patient
2. Palliative care and hospice
3. Euthanasia and assisted suicide: concepts and types
4. The normative framework
5. Arguments and philosophical perspectives

The terminally ill patient

Incurable, terminal, dying

Term	Meaning	Example
Incurable	Cannot be removed, but may be controlled for years	Heart failure, multiple sclerosis
Terminal	Progressive; treatment no longer changes the prognosis for length of life	Metastatic cancer after exhausted therapy
Dying	Irreversible failure of vital functions has begun	The last hours or days

Medical Institutions Act, § 1, item 4 (Additional Provisions): terminally ill patients have progressive, incurable diseases in which treatment does not change the unfavourable prognosis for length of life.

Ethical problems at the end of life

Truth and prognosis

How much, when and to whom; the family's request to hide the diagnosis.

Proportionality

Therapeutic obstinacy versus abandonment; futility.

Refusal

A capable patient refuses dialysis, ventilation or surgery.

Resuscitation

Do-not-resuscitate decisions, made before the crisis.

Incapacity

Who speaks for the patient: prior wishes, relatives, best interests.

Justice

Access to care regardless of diagnosis, place or income.

The terminally ill patient keeps all patient rights

Provision	Content
Health Act, Art. 90	Refusal of offered care or of its continuation at any time; recorded with signatures.
Health Act, Art. 91	Care against the patient's will only in cases set by law.
Health Act, Art. 92(2), (4)	Right not to be told the prognosis; written authorisation of a person to be informed instead.
Code of Ethics, Art. 29	On refusal from ignorance of the dangers, the physician explains the prognosis without treatment.

The right not to know belongs to the patient, not to the family.

Withholding, withdrawing and proportionality

Withholding = withdrawing

Not starting and stopping can have the same ethical basis. A trial that fails its agreed goal need not continue.

Do not resuscitate

Only CPR at arrest. Pain relief, oxygen, fluids and presence continue.

Proportionate treatment

Benefit matches burden for **this** patient's goals. Futility is a clinical judgement, not "a life not worth living".

Nutrition and hydration

Medical interventions with their own indications; never "letting someone starve".

Case 1 · The seventh dialysis

A 60-year-old man with diabetes and severe kidney failure stops coming after six haemodialysis sessions. On the phone he says he does not want the rest of his life to revolve around dialysis. At his last visit he was oriented, but low in mood, and said he felt a burden to his family. He knows that stopping will probably lead to death.

- Is stopping dialysis euthanasia?
- What must be clarified before the refusal is accepted?
- What duties remain if he confirms it with capacity?

Palliative care and hospice

What palliative care is

WHO (2002): an approach that improves the quality of life of patients and their families facing life-threatening illness, through the prevention and relief of suffering by early identification, impeccable assessment and treatment of pain and other problems, physical, psychosocial and spiritual.

- Not only cancer, not only the last days: it starts according to need.
- It may run alongside treatment aimed at the disease.
- Early palliative care improved quality of life and mood (Temel et al., 2010).

Goals of palliative care

1

**Relief of pain
and symptoms**

2

**Neither hastens
nor postpones
death**

3

**Psychosocial
and spiritual
support**

4

**Living as
actively as
possible**

5

**Support for the
family, also in
grief**

Health Act, Art. 95(2): the goal is to maintain quality of life by reducing or removing manifestations of the disease and its unfavourable psychological and social effects.

Palliative care is active treatment, not the end of treatment.

Total pain and three overlapping terms

Term	Scope	When
Palliative care	Relief of suffering in any serious, life-limiting illness	From diagnosis, as needed
End-of-life care	Comfort and planning when death is expected soon	Last months, weeks, days
Hospice care	Organised palliative care in advanced disease, with family support	Limited prognosis; comfort goal

Total pain (Cicely Saunders): physical, psychological, social and spiritual suffering reinforce one another.

Hospice and palliative care in Bulgarian law

Health Act, Arts. 95–96

Right to palliative care in incurable disease with unfavourable prognosis. Monitoring, relief of pain and distress, moral support. Provided by the GP, outpatient and hospital care, and hospices. Requirements: by ordinance (Art. 96(3)).

Medical Institutions Act, Arts. 10, 28

The hospice is a medical institution where specialists give palliative care to terminally ill patients; it may also offer social services.

The gap: the ordinance under Art. 96(3) was never issued; there is no clear mechanism for the scope and control of palliative care (Ilieva and Musurlieva, 2023).

Opioids and the rule of double effect

1

The act is good or neutral

2

Only the good effect is intended

3

The harm is not the means

4

A proportionate reason

- Titrated opioids are an ordinary medical duty; they do not routinely hasten death.
- Intention shows in indication, dose, pace of increase, monitoring and records.
- A lethal dose given to cause death is not “pain relief”.

Palliative (terminal) sedation

Indication

A refractory symptom: no effective treatment within an acceptable time or burden. Last resort, after specialist review.

Consent and records

Informed decision; the symptom, earlier attempts, the regimen and monitoring are documented.

Proportionality

The lowest level of sedation that relieves; light or intermittent first. Continuous deep sedation only in the last hours or days.

Not euthanasia

The aim is relief; death is neither the goal nor the means.

Case 2 · Increasing the dose

A 74-year-old man with metastatic melanoma is bedbound and takes an opioid on a fixed schedule, but for two days has had almost constant pain and painful episodes when moved. On a home visit he asks for “the strongest dose, whatever happens”. His daughter objects: she fears morphine will stop his breathing and hasten his death.

- Why is “pain or a lethal dose” a false choice?
- What assessment comes before changing the analgesia?
- How do you explain double effect to the daughter?

Euthanasia and assisted suicide: concepts and types

Who performs the last act?

Practice	Last act	Intention	Cause of death
Euthanasia	Physician	To cause death at a valid request	The substance given
Physician-assisted suicide	Patient, with means from a physician	The patient ends their life	The substance taken
Refusal / withdrawal	No lethal act	Respect refusal; stop futile care	The underlying disease
Palliative sedation	Team, proportionate dose	Relieve refractory suffering	The progressing disease

Etymology: Greek *eu* (good) + *thanatos* (death). “A good death” is wider than euthanasia.

Classical types of euthanasia

Criterion	Types	Assessment today
By the act	Active: an act causes death. Passive: treatment is withheld or withdrawn	“Passive euthanasia” is misleading: proportionate withdrawal is not euthanasia
By the will	Voluntary (at request), non-voluntary (no capacity), involuntary (against the will)	Only voluntary can be euthanasia; involuntary is murder
By intention	Direct: death intended. Indirect: death foreseen, not intended	Indirect = double effect; not euthanasia
By the actor	Euthanasia vs (physician-)assisted suicide	Decided by who performs the last act

What is not euthanasia

Not euthanasia

- Refusal by a capable patient.
- Withholding or withdrawing futile treatment.
- Titrated pain relief.
- Proportionate palliative sedation.

Four tests

- What is done?
- What is intended?
- What causes death?
- Does care continue?

Relatives cannot request euthanasia on behalf of a patient without capacity.

“I want to die”: a request that needs an answer

1

Ask what makes
life unbearable

2

Assess
symptoms,
mood, capacity,
risk

3

Treat what can
be treated

4

Explain the
legal limits

5

Promise not to
abandon

“I cannot perform or help with this, but I want to understand what makes your situation unbearable and how we can help you now.”

The normative framework

Bulgaria: euthanasia is prohibited

Act	Provision
Constitution, Art. 28	Everyone has the right to life; an attack on it is punished as the gravest crime.
Health Act, Art. 97	Euthanasia is not applied on the territory of Bulgaria.
Code of Ethics, Arts. 30, 31	The physician has no right to end the patient's life; must do everything possible for a life without pain, with dignity.
Criminal Code, Arts. 115, 116	No privileged "killing on request": deliberate killing is murder; consent is no defence.
Criminal Code, Art. 127	Assisting or persuading another to commit suicide is a crime if suicide or an attempt follows.

The prohibition does not cancel the rights to refuse treatment and to palliative care.

Council of Europe, Strasbourg and the WMA

Source	Key point
Pretty v UK (2002)	Art. 2 confers no right to die; a blanket ban can be proportionate.
Haas v Switzerland (2011)	Deciding how and when to die falls under Art. 8; states have a wide margin.
Lambert v France (GC, 2015)	Withdrawing nutrition in a vegetative state did not breach Art. 2.
Mortier v Belgium (2022)	Art. 2 breached by non-independent review, not by the euthanasia law.
Karsai v Hungary (2024)	No duty to allow assisted dying; palliative care is essential.
Oviedo, Art. 9; PACE Res. 1859 (2012)	Prior wishes are taken into account; euthanasia must remain prohibited.
WMA (Tbilisi 2019; Venice 2022)	Firmly opposed to euthanasia and PAS; palliative care is good medicine.

A short global overview (October 2026)

Model	Jurisdictions
Euthanasia and assisted suicide	Netherlands, Belgium, Luxembourg, Canada (MAID), Spain, Colombia, New Zealand, Australian states and ACT
Euthanasia only	Uruguay (2025)
Assisted suicide only	Switzerland (no selfish motive), Oregon and over ten US jurisdictions incl. New York (2026), Austria, Germany and Italy (by court rulings)
Debated or not in force	France (2026, constitutional review), England and Wales, Scotland (rejected), Portugal, Slovenia (referendum “no”)
Prohibited	Bulgaria and most Council of Europe states

Where legal: 0.3–4.6% of deaths, over 70% cancer; pain is rarely the main motive (Emanuel et al., 2016).

Arguments and philosophical perspectives

Arguments for and against

For legalisation

- Autonomy over one's own death.
- Compassion: not all suffering can be relieved.
- Dignity as living by one's values.
- Transparency instead of hidden practice.

Against

- The inherent value of life; the healer's role.
- Uncertain prognosis, irreversible act.
- Pressure on the old, poor and disabled.
- The slippery slope; palliative care first.

“Dignity” and “do no harm” serve both sides: define them first.

Philosophical perspectives

Perspective	Core claim for end-of-life decisions
Autonomy (Mill)	A capable person decides about their own life; a right to refuse is not a right to oblige another to kill.
Sanctity of life	Life is inviolable and must not be intentionally ended; it does not demand endless treatment.
Kant	Persons have dignity, not a price; ending life to escape suffering treats oneself as a mere means.
Utilitarianism	Minimise suffering (Singer); but consider trust in medicine and risk to the vulnerable.
Slippery slope	Logical: the reasons extend beyond terminal illness. Empirical: data are contested.
Hippocratic tradition	"I will not give a deadly drug to anybody who asks for it."

Case 3 · Documents for abroad

A 60-year-old woman with amyotrophic lateral sclerosis has full capacity and communicates through an eye-tracking device. She wants to apply to an organisation in a country that allows assisted suicide. She asks her neurologist for a detailed medical summary, translated results and direct contact with the organisation.

- Where is the line between her right to her records and facilitating suicide?
- What should the neurologist offer her besides a refusal?
- When should legal and ethics advice be sought?

When cure is no longer possible, care is still
owed.

Not abandonment, not obstinacy: relief, honesty and presence.